

From: (b) (6) (CBER)

Sent: Friday, November 27, 2020 3:53 PM

To: Harkins Tull, Elisa <Elisa.HarkinsTull@pfizer.com>; Devlin, Carmel M <Carmel.Devlin@pfizer.com>

Cc: (b) (6) @fda.hhs.gov>

Subject: EUA 27034: IR with 5 clinical questions

Elisa,

The clinical team has five questions for Pfizer and they requested a response by COB Monday.

- 1- Regarding tables 26 and 27 from the 508 compliant tables document you submitted in Microsoft Word format: *Table 26: Percentage of subjects reporting the occurrence SAEs from Dose 1 to 1 Month After Dose 2 classified by MedDRA Primary System Organ Class and Preferred Term – Safety Population** and *Table 27: Percentage of subjects reporting the occurrence SAEs from Dose 1 to 1 Month After Dose 2 classified by MedDRA Primary System Organ Class and Preferred Term, by Baseline SARS-CoV-2 Status - Safety Population*Baseline SARS-CoV-2 Status: Positive*. Please clarify the cutoff date for the data summarized in these tables. Please also clarify if the follow-up period is one month follow-up post dose 2 or is it up to the November 14, 2020, cutoff date. We had requested all safety information available up to the November 14, 2020, cutoff. Please confirm and comment.
- 2- Regarding the 58 subjects who received a higher dose due to a medication error, please provide us with a listing of these subjects and indicate whether we can identify them with any variable in the database. We cannot identify a 50 µg dose in EX or ADSL variables. The ADAE we have AE Category Medication error, but the Incorrect dosages do not specify dose actually given. Could you please guide us to where we can find the variable or if there is a flag to use to identify these subjects?
- 3- Please provide a summary table by study arm of all participants who had a potential COVID-19 illness that was not confirmed to be COVID-19. Please include numbers and percentages of subjects in each study arm who had a potential COVID-19 illness at any time during follow-up, during the 7 days post-dose 1, and during the 7 days post-dose 2. Please include a listing of all these participants by study arm as well.
- 4- Regarding the efficacy outcomes stratified by risk factors, with the purpose of obtaining more COVID-19 specific risks and meaningful information, we request that you provide analyses by each of the following risk factors for COVID-19 disease. We had agreed on reporting efficacy outcomes by obesity as a risk factor independently of others (35% of the population). You may also report efficacy outcomes by number of subjects who have 1 or more comorbidities that increase the risk of severe COVID-19 disease: defined as patients who meet at least one of the following criteria:
 - >=65 yo
 - Chronic lung disease (e.g., emphysema and chronic bronchitis, idiopathic pulmonary fibrosis, and cystic fibrosis) or moderate to severe asthma
 - Significant cardiac disease (e.g., heart failure, coronary artery disease, congenital heart disease, cardiomyopathies, and pulmonary hypertension)

- Hypertension
- Obesity (body mass index ≥ 30 kg/m²)
- Diabetes (Type 1, Type 2 or gestational)

5- Please clarify whether the ~8,000 subjects in the reactogenicity subset are indeed the first ~8,000 enrolled subjects. In the protocol, the plan was to obtain reactogenicity data from the first 6,000 subjects enrolled and included the safety population. Please confirm.

Regards,

(b) (6)

Please confirm receipt of this e-mail and let us know if you have any questions.

(b) (6)

**Food and Drug Administration
Center for Biologics Evaluation & Research
Office of Vaccines Research & Review**

(b) (6)

Tel: (b) (6)

(b) (6) [@fda.hhs.gov](mailto:(b) (6)@fda.hhs.gov)



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